MANNSETU

Competitive Landscape & Digital Mental-Health Front Door Research

Comparative analysis of Wysa DRA, Limbic Access, Tele-MANAS, YourDOST and adjacent models

Research brief | September 2026

Key conclusion

The broad concept already exists internationally under the category of a “digital front door” for mental healthcare. However, MannSetu can still be differentiated by making the LLM an information-extraction layer only, placing routing decisions inside a clinician-authored deterministic protocol, and integrating that protocol with India-specific university, hospital, helpline and community care pathways.

Scope note: This is a competitive and product-landscape research brief, not clinical or regulatory advice. Product capabilities and regulatory status can change; claims should be re-verified before public pitch, procurement, or clinical deployment.

1. Executive Summary

MannSetu is best understood as a **digital mental-health front door**: a conversational intake and navigation layer that helps a person reach an appropriate existing source of support. The market category is real and already deployed, particularly in the UK NHS. The closest technical comparators found in this review are Wysa Digital Referral Assistant (DRA) and Limbic Access. In India, Tele-MANAS is the strongest infrastructure analogue, while YourDOST is a strong university-access analogue.

- The core idea - conversational intake, structured screening, triage and routing - is not globally novel.
- Wysa DRA is the closest architectural comparator because it uses a conversational AI interface, screening data, transparent conditional logic and clinician review, while explicitly avoiding diagnosis or replacement of clinical judgement.
- Limbic Access is the strongest evidence-backed digital-front-door benchmark and is a UKCA Class IIa medical device used in NHS Talking Therapies.
- Tele-MANAS demonstrates that India already has a large-scale public mental-health access and referral network that MannSetu could complement rather than replicate.
- YourDOST demonstrates institutional demand for confidential, digitally accessible mental-health support in Indian universities.
- The defensible MannSetu proposition is therefore not “AI therapy” or “AI diagnosis”; it is a safety-first navigation layer in which AI understands conversation, while clinician-designed rules determine routing.

2. MannSetu Model Being Evaluated

The proposed user journey has three steps:

1. Talk - A pseudonymous user begins a private Hindi/English conversation without needing to enter a name.
2. Triage - The conversation naturally incorporates structured screening questions inspired by validated mental-health screening formats, while the AI converts free text into structured signals.
3. Route - A deterministic safety protocol maps structured signals to self-help resources, a counsellor referral, or urgent crisis resources.

Architectural separation

Layer	Primary role	May do	Must not do
1. LLM	Conversation + extraction	Convert free text into structured observations and ask clarifying questions	Make a clinical judgement or independently choose a care pathway
2. Safety protocol	Deterministic routing	Apply clinician-authored rules to structured signals	Invent or improvise clinical criteria
3. Human care	Clinical support	Counselling, psychiatric assessment, crisis response or other real care	Be replaced by the chatbot

3. Existing Category: “Digital Front Door”

The term “digital front door” is already used in UK health-technology evaluation for systems that collect referral information before a mental-health assessment. A 2026 NIHR/NICE-linked health technology assessment evaluated Limbic Access, Wysa DRA, Censeo Digital and AskFirst in NHS Talking Therapies. It

found relevant evidence only for Limbic Access and Wysa DRA, with the Limbic evidence base stronger overall.

Implication for MannSetu: Avoid claiming the invention of AI-based mental-health triage or the world's first conversational mental-health front door. Position the innovation at the architectural, safety, localization and network-integration layers.

4. Closest Platforms

4.1 Wysa Digital Referral Assistant (DRA)

Wysa DRA is the closest architectural comparator identified in this review. Wysa describes it as an AI-driven conversational agent that collects demographic and screening information for transfer to electronic patient-record systems for clinician review. Wysa states that DRA uses transparent conditional logic, surfaces priority flags, and does not suggest diagnoses, provide real-time medical advice, or replace clinical judgement.

- Very close to MannSetu: conversational intake, structured screening, conditional logic, risk/priority flags and clinician involvement.
- Important difference: MannSetu can make the LLM-to-protocol boundary more explicit by formally constraining the LLM to extraction and clarification only.
- Competitive lesson: deterministic or conditional routing is not, by itself, novel; the value must come from the complete safety architecture, India-specific routing graph and evidence strategy.

4.2 Limbic Access

Limbic Access is a major benchmark for digitally assisted mental-health intake. The 2026 health technology assessment describes it as a UKCA Class IIa medical device currently used in NHS Talking Therapies. NICE material describes it as an AI chatbot intended to guide people to the right service by collecting eligibility information, demographics and presenting symptoms. Compared with MannSetu, Limbic is more willing to use algorithmic/AI clinical decision support, whereas MannSetu proposes a stricter "AI extracts, protocol decides" boundary.

- Strongest benchmark for evidence, regulation and NHS integration among the systems reviewed.
- Published evidence suggests increased referrals after Limbic implementation and improved access for some underrepresented groups; however, the 2026 assessment still identified significant evidence gaps.
- Competitive lesson: MannSetu should plan validation around accuracy, acceptability, equity, assessment-time reduction and referral appropriateness - not just chatbot quality.

4.3 Tele-MANAS (India)

Tele-MANAS is the closest Indian infrastructure analogue. According to India's Ministry of Health and Family Welfare, as of 3 March 2026 it had 53 cells across all 36 States/UTs, supported 20 languages, and had handled more than 34.34 lakh calls since inception. The same government update reported 47,487 referrals for non-emergency in-person services and 9,590 referrals for further detailed clinical evaluation.

- Tele-MANAS proves the need for a scalable entry-and-referral layer in India.
- MannSetu should be designed as complementary infrastructure that can route toward public systems such as Tele-MANAS, rather than positioning itself as a replacement.
- The largest difference is interface and automation: Tele-MANAS is fundamentally a public tele-mental-health service, whereas MannSetu is proposed as a web-first conversational intake and routing layer.

4.4 YourDOST (India / universities)

YourDOST is a strong analogue for the university-access part of MannSetu. IIT Kanpur currently describes YourDOST as a confidential, free, 24x7 online counselling option for students with trained professionals, expert browsing, self-help tools and self-assessments. IISc similarly promotes YourDOST as an online emotional-wellness platform offering professional counselling with privacy, confidentiality and anonymity.

- Validates institutional demand for private, digitally accessible mental-health support.
- Differs from MannSetu because YourDOST primarily provides access to its professional support network, whereas MannSetu is envisioned as neutral routing infrastructure spanning multiple existing providers/resources.

5. Competitive Comparison

Capability	Wysa DRA	Limbic	Tele-MANAS	YourDOST	MannSetu
Conversational intake	Yes	Yes	Human/tele-led	Human expert access	Yes
Structured mental-health screening	Yes	Yes	Protocol/service dependent	Self-assessments	Yes
Free-text AI understanding	Yes	Yes	Not the core interface	Not the core triage layer	Yes
AI diagnosis / predictive clinical decision	No diagnosis	Yes / decision support	No	No	No - intentionally
Deterministic / conditional protocol	Yes	Mixed / algorithmic	Protocol-based service	Not core differentiator	Core architecture
Crisis / risk escalation	Yes	Yes	Yes	Yes	Yes
Routes into professional care	Yes	Yes	Yes	Direct access	Yes
India-first	Company origin India; DRA benchmark is NHS	No	Yes	Yes	Yes
University-first MVP	No	No	No	Yes	Yes
Pseudonymous-first design	Deployment dependent	Deployment dependent	Confidential service	Anonymous/ confidential options	Core MVP goal
Network across independent local resources	Limited by deployment	NHS pathway-specific	National public network	Own/partner support ecosystem	Long-term goal

6. What MannSetu Can and Cannot Claim

Claims to avoid

- “World’s first AI mental-health triage platform.”
- “First conversational mental-health referral system.”
- “First system to use rules rather than AI for mental-health routing.”
- Any claim that the system diagnoses, treats, or guarantees correct clinical triage.

More defensible positioning

“MannSetu is a safety-first digital front door to India’s existing mental-health ecosystem. AI is used to understand conversation and extract structured signals; clinician-authored protocols determine routing to self-help, professional care or crisis services.”

7. Where the Product Opportunity Still Exists

4. India-specific care orchestration: Build a live, location-aware directory of university counselling cells, hospitals, Tele-MANAS, verified counsellors, NGOs and crisis pathways instead of owning all care delivery.
5. Hard safety boundary: Treat the LLM as a constrained parser/conversational interface, with deterministic policy enforcement and an explicit “ambiguous -> ask, never guess” mechanism.
6. University deployment model: Offer privacy-preserving aggregate analytics to institutions while keeping individual conversations inaccessible to the university.
7. Bilingual and culturally adapted intake: Begin with Hindi + English and validate phrasing, code-switching, euphemisms and local expressions rather than merely translating English questionnaires.
8. Interoperability: Make MannSetu a routing layer that can hand users off to existing systems instead of becoming another closed counselling marketplace.
9. Evidence-first rollout: Predefine safety metrics and compare routing decisions against clinician judgement before broad deployment.

8. Recommended Validation Framework for an MVP

The 2026 digital-front-door evidence review highlights that the field still has meaningful evidence gaps. MannSetu can use that as a design advantage by building evaluation into the product from the pilot stage.

Metric	What to measure	Why it matters
Routing agreement	Agreement between protocol outcome and blinded clinician judgement	Primary safety/appropriateness signal
Critical-risk sensitivity	Whether high-risk cases are consistently escalated	False negatives are the most safety-critical failure

Clarification rate	How often ambiguity triggers a follow-up question	Tests the “ask rather than guess” design
Completion rate	Users who finish intake / reach a destination	Measures friction and usability
Time to appropriate care	Time from first contact to relevant resource/referral	Tests the core value proposition
Equity	Performance by language, gender, demographic and access group where ethical/appropriate	Detects uneven performance
Counsellor workload	Assessment/admin time before and after the system	Measures institutional benefit
User trust & acceptability	Perceived privacy, clarity, usefulness and willingness to reuse	Necessary for real adoption

9. Strategic Conclusion

MannSetu should not be pitched as a new category. The category already exists, and Wysa DRA plus Limbic Access provide direct precedent. Instead, the opportunity is to build a carefully localized, safety-constrained and interoperable version for India, starting in universities.

The competitive landscape is therefore encouraging rather than disqualifying:

- Wysa and Limbic demonstrate that conversational digital front doors are technically and operationally feasible.
- Tele-MANAS demonstrates that India has a substantial public mental-health access and referral infrastructure.
- YourDOST demonstrates that Indian universities already deploy digital mental-health support services.
- The open product question is whether MannSetu can connect these types of resources safely, neutrally and measurably better than users can navigate them themselves.

10. Sources & Further Reading

1. **NIHR / NCBI Bookshelf (2026), Digital front door technologies to pre-assess people before assessment for NHS Talking Therapies - Scientific summary.**
   https://www.ncbi.nlm.nih.gov/books/NBK623344/
2. **NIHR / NCBI Bookshelf (2026), Overview of the technology.**
   https://www.ncbi.nlm.nih.gov/books/NBK623349/
3. **NIHR / NCBI Bookshelf (2026), Information about the digital front door technologies.**
   https://www.ncbi.nlm.nih.gov/books/NBK623335/
4. **PubMed (2026), Digital front door technologies early value assessment.**
   https://pubmed.ncbi.nlm.nih.gov/42366809/

5. **NICE, HTG756: Digital front door technologies for NHS Talking Therapies.** https://www.nice.org.uk/guidance/htg756
6. **Wysa, Digital Referral Assistant FAQ / intended use.** https://www.wysa.com/faq-vita-etriage
7. **Wysa, DRA product demo / intended use information.** https://www.wysa.com/wysa-demo-dra
8. **Government of India, PIB (23 Mar 2026), Update on Tele-MANAS.** https://www.pib.gov.in/PressReleasePage.aspx?PRID=2243766&lang=2&reg=3
9. **IIT Kanpur Center for Mental Health and Wellbeing, External Counselling.** https://iitk.ac.in/counsel/cmhw/pages/external-counselling.php
10. **IISc Wellness Centre, YourDOST / online counselling.** https://wellness.iisc.ac.in/index.php/your-dost/